Supplementary Table 3: Template risk of bias assessment form

Study ID: Lauridsen et al., 2018

Item	Considerations	Authors' judgement	Support for judgement
Representativeness of the GDM cohort	• the subjects covered in this study could be sufficiently different from your population to cause concern • your local setting is likely to differ much from that of the study	Low risk	DNBC cohort
Unexposed cohort drawn from the same population	• were all the subjects drawn from the same base cohort?	Low risk	same cohort for control selection
Objective ascertainment of exposure	• did they use subjective or objective measurements • were all the subjects classified into exposure groups using the same procedure	Low risk	register based + self-reported information
No contamination of exposure status with pre-existing diabetes	• do the measurements truly reflect what you want them to (have they been validated) • does the exposed and the unexposed cohort exclude patients with pre-existing diabetes	Low risk	clear stratification of T1DM, T2DM and GDM
Adjustment for covariates in the analysis	• do the authors identify and adjust for confounders in their analysis	Low risk	adjusted for maternal age at menarche, maternal age at birth,socioeconomic staus, cohabitation, parity and maternal pre-pregnancy BMI
Blind outcome measurement	• were the subjects and/or the outcome assessor blinded to exposure (does this matter)	High risk	no report of blinding of the researchers
Objective ascertainment of outcomes	• were the measurement methods similar in the different groups	High risk	questionnaire and self-reports

	• did they use subjective or objective measurements • do the outcomes truly reflect what you want them to (have they been validated)		
Imprecision of outcome timing accounted for in the analysis	• were the age at maturation events known exactly • If not, did the authors use appropriate statistical approach such as interval censored regression analysis for time-to-event data / Did the authors consider multiple longitudinal outcome measurements	Low risk	interval censoring
Other causes of precocious puberty investigated	• did the authors record and report non-idiopathic causes of sex steroid producing tumours or hypothalamic abnormalities.	High risk	no report
Sufficient follow-up time	• was the follow-up of the offspring long enough for the range of maturational events to be recorded? • did the authors report median follow-up period?	Low risk	from the age of 11.5, children were followed up until they all they reached full sexual maturation or turned 18
Loss to follow-up minimal or nil	• proportion willing to report outcomes • proportion dropped out of the cohort	High risk	only 71% of the children provided information on pubertal development